
Breath as Regulation

Your breathing is either helping your body
recover or keeping it locked up

NATE RATCLIFF, LMT

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HEALING HANDS BY NATE

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Continuity Member Edition — Month 2

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Introduction

A BETWEEN-SESSIONS MODULE ON THE cheapest, most available reset tool you own — and how most people use it wrong.

I want you to watch yourself breathe for ten seconds right now. Don't change anything. Just watch. Are your shoulders moving? Yeah. That's what we're fixing today.

Last month was about noticing where stress lives in your body. This month is about the most leveraged single intervention you can make to interrupt the stress cascade: how you breathe.

Most people don't think about breathing. It happens automatically. They assume it's fine because they're alive. That assumption costs them more than they realize.

Your breathing pattern is either signaling "we're safe, you can recover" to your nervous system, or it's signaling "we're under threat, stay alert." Most people, most of the time, are unknowingly signaling the second one — even when nothing is actually wrong.

This module is about flipping that signal.

CHAPTER 1

What's actually happening when you breathe

YOUR DIAPHRAGM IS THE PRIMARY BREATHING muscle. It's a large dome-shaped muscle that separates your chest from your abdomen. When you breathe deeply and correctly, the diaphragm contracts downward, expanding the lower lungs and pushing your belly outward.

This is what babies do. Watch a sleeping baby breathe — their belly rises and falls. Their chest barely moves. That's healthy, regulated breathing.

Somewhere between childhood and adulthood, most people lose this. Stress, posture, anxiety, and habits gradually shift the breathing pattern upward. The diaphragm stops doing most of the work. The “accessory” muscles — the ones around the upper chest, shoulders, and neck — take over.

These accessory muscles aren't designed for 22,000 breaths a day. They're designed for high-output situations: sprinting, heavy lifting, fighting. When they become your default breathing pattern, they're chronic-

ally overworked. Result: tight neck, tight shoulders, tight chest, shallow oxygen delivery, and constant low-grade nervous-system arousal.

CHAPTER 2

How to tell which pattern you use

THE TEST I GAVE YOU ABOVE IS THE SIMPLEST one. Watch yourself breathe normally for 10 seconds. Don't change anything.

If your shoulders rise and fall with each breath, you're shoulder-breathing. If your upper chest expands but your belly stays still, you're chest-breathing. Both are accessory-muscle dominant. Both are the stress pattern.

The healthy pattern: belly rises on inhale, belly falls on exhale. Shoulders stay still. Chest moves only minimally.

For a more sensitive test:

Lie flat on your back. Place one hand on your chest, one on your belly.

Breathe normally for a minute. Which hand is moving?

Most adults find that the chest hand moves significantly more than the belly hand. If that's you, you're a chest-breather, probably most of the time, even

though lying down should naturally promote diaphragmatic breathing.

This isn't a moral failing. It's an adapted pattern. The fix is straightforward.

CHAPTER 3

Why this matters more than you'd think

YOUR BREATHING PATTERN IS ONE OF THE ONLY direct, voluntary inputs you have to your autonomic nervous system. The autonomic system runs most of your body without conscious control — heart rate, digestion, blood pressure, immune function. You can't directly will your heart to slow down. But you CAN change your breathing pattern. And when you do, your nervous system responds.

Specifically:

- **Slow, deep diaphragmatic breathing → parasympathetic activation** (rest, digest, recover)
- **Rapid, shallow upper-chest breathing → sympathetic activation** (fight, flight, alertness)

Your nervous system reads your breath as a primary signal about what state you're in. Breathe shallow → it concludes you're under stress → it keeps the stress cascade running.

Breathe deep → it concludes you're safe → it downshifts.

This works automatically and reliably. It's not a placebo. It's biology.

The implication: by deliberately practicing diaphragmatic breathing, you're directly intervening in your nervous system's threat response. You're sending a signal that says "stand down."

CHAPTER 4

The exhale matters more than the inhale

MOST BREATHWORK INSTRUCTION FOCUSES ON the inhale. “Take a deep breath in.” “Fill your lungs.” That’s incomplete.

The exhale is where the magic happens. The exhale phase activates the parasympathetic nervous system far more strongly than the inhale.

Here’s why: when you exhale, your heart rate naturally slows slightly (this is called respiratory sinus arrhythmia, and it’s a sign of healthy nervous-system regulation). When you inhale, it speeds up slightly. So a longer exhale than inhale creates a bigger drop in heart rate per breath, which signals “we’re safe” more clearly to the nervous system.

This is why most effective breathwork patterns include a longer exhale than inhale:

- 4-7-8 breathing (inhale 4, hold 7, exhale 8)
- Box breathing variants
- “Coherent breathing” patterns
- Pranayama techniques

All of them rely on exhale dominance to drive the parasympathetic shift.

CHAPTER 5

The pattern I recommend

DON'T OVERCOMPLICATE THIS. FOR MOST people, this works as well as anything more elaborate:

Inhale through your nose for 4 counts. Exhale through your mouth for 6 counts.

That's it. 4 in, 6 out. Practice this for 5 minutes a day.

The 6-count exhale is the key. The longer-than-inhale exhale is what drives the nervous system shift.

If 4-6 feels too fast or too slow, adjust to whatever counts feel natural for you. The pattern doesn't have to be exactly 4-6 — it just has to have the exhale longer than the inhale.

For people who like more structure, the 4-7-8 pattern is the classic:

Inhale 4 — hold 7 — exhale 8.

That works too. Slightly more intense. Use it before bed if you have trouble falling asleep — it's reliably effective for that.

CHAPTER 6

How to actually practice this without it feeling like homework

MOST “BREATHWORK PRACTICE” RECOMMENDATIONS are unrealistic. “Sit in a quiet space for 20 minutes and breathe mindfully.” Few people actually do that consistently.

Here’s how to make it stick:

Method 1 — Anchor it to existing routines

Pick a thing you already do every day. Breathe during that thing.

- 5 minutes of slow breathing while your morning coffee brews
- 5 minutes during your commute (if you’re not driving — passenger only)
- 5 minutes after dinner before doing anything else
- 5 minutes while lying in bed before sleep

The “5 minutes alongside something existing” version succeeds where “5 dedicated minutes” often fails.

Method 2 — Micro-doses throughout the day

Instead of one 5-minute block, do 10 slow breaths (about 60-90 seconds) multiple times throughout the day.

- 10 breaths when you wake up
- 10 breaths before a meeting
- 10 breaths after a stressful conversation
- 10 breaths before bed

This adds up to 5+ minutes total without ever feeling like a dedicated practice block.

Method 3 — Combine with body scan

This pairs naturally with last month’s awareness practice. When you scan and find tension, do 5 slow breaths into that area. The combined effect is greater than either alone.

CHAPTER 7

What to expect

BY WEEK 2 OF CONSISTENT PRACTICE (ANY OF the methods above):

- Slower heart rate at rest
- Easier transition into sleep
- Less feeling of “wired but tired”
- More patience throughout the day
- Slightly deeper breath even when you’re not actively practicing

By month 2:

- Your default breath pattern starts shifting toward belly-dominant
- Catching yourself shoulder-breathing during stressful moments becomes automatic
- Brief breathing resets during the day become a natural habit
- Stress response is noticeably less reactive

This isn’t dramatic. It’s small, daily, structural. But the cumulative effect on your nervous system is significant.

CHAPTER 8

The connection to bodywork

HERE'S WHY THIS MATTERS FOR THE WORK I DO on the table:

When I see a client who's chronically chest-breathing, I can release their neck and shoulders during a session. They'll leave looser than they came in. Within 3-4 days, the tension is usually back, because the shoulder-breathing is continuously re-tightening the muscles I just released.

When that same client has been practicing diaphragmatic breathing for 30 days, the work I do on the table holds significantly longer. The breath pattern has shifted. The continuous re-tightening has slowed. The release sticks.

In other words: breathwork makes my work last. Skip the breathwork and you're paying for sessions that come undone in a week. Practice the breathwork and the same sessions hold for a month or more.

This is one of the highest-ROI things you can do between sessions.

CHAPTER 9

When this won't work

IF YOU HAVE A RESPIRATORY CONDITION (asthma, COPD, severe anxiety with breath-related panic), talk to your physician before doing structured breathwork. The patterns can sometimes feel uncomfortable for people with these conditions and need to be modified.

Otherwise, this is essentially risk-free. Worst case, you spent 5 minutes a day breathing.

CHAPTER 10

Closing

FIVE MINUTES. BELLY MOVES, SHOULDERS don't. Do that every day this month and you'll feel a difference you can't unfeel.

Breathing is the most leveraged single intervention I know for stress, tension, sleep, and recovery. It's free. It's available anytime. It works through measurable biology, not vague self-help mechanisms.

The barrier isn't knowledge. It's consistency.

Start tomorrow morning. 5 minutes. Hand on belly. Inhale 4, exhale 6. That's the assignment.

The companion workbook has a structured 7-day breath practice tracker, a breath-pattern self-assessment, and three reflection prompts to help you notice how your breath changes throughout your day.

— Nate



This book was crafted by Nate Ratcliff,
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